



GLOBAL HUMANITARIAN OVERVIEW 2026

**MID-YEAR
REVIEW**

JUNE 2026



Bamyan Province, Afghanistan

Tahera, a hardworking farmer, is harvesting her wheat field after a year of dedicated effort.
FAO/Hashim Azizi.

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Mid-Year Review of the Global Humanitarian Overview: Delivering for People in Crisis against the Odds

People in need

252.1 M

People targeted

143.2 M

People prioritized

88.7 M

As of 31 May 2026:

GHO requirements

\$33.66 BN

GHO funding

\$8.21 BN

GHO coverage

24.42%



GHO prioritized requirements

\$24 BN

GHO prioritized funding

\$7.89 BN

GHO prioritized coverage

32.8%



Bulukat Transit Centre, Malakal, Upper Nile State, South Sudan

USG Tom Fletcher and UN Resident Coordinator Anita Kiki Gbeho speak with Naketta, 38, who returned to South Sudan after fleeing conflict in Sudan. She describes a journey marked by violence and loss as over 1.3 million people have crossed the border since April 2023. Now in Bulukat, she is rebuilding through small a small bread-baking business, but still struggles to secure food, safety, and education for her children.

UNOCHA/Adedeji Ademigbuji



Foreword by the Emergency Relief Coordinator

When I launched the 2026 Global Humanitarian Overview in December 2025, we set out a rigorous, evidence-based plan to reach 87 million people in crisis who were most at risk of death.

That plan was built on excruciating choices forced on us by deep funding cuts — choices no humanitarian ever wants to make; choices that leave millions of people without any realistic hope that help will come. And as we release this GHO Mid-Year Review, even more people require our support than at the beginning of the year.

But people in crisis do not experience these cuts as numbers.

If you are a mother trying to find food or treatment for your children, you are walking much further today than you were a year ago. Clinics have closed. Distribution sites are fewer and further between. And when you arrive, you do not know whether you will receive help. The poet Peter Kidi captures this reality in Kakuma:

Category 3.

No food ration. No cooking oil.

No explanation.

Just a screen that says: Not today. Not you.

As we approach the middle of the year, humanitarians have continued to work in the toughest places on earth to deliver. We are focusing what we have on the areas of greatest need, because we no longer have the resources to do more. And this is what impartiality requires of us—to respond to the most urgent cases of distress first, on the basis of need alone.

Yet while we work to deliver, others are working to destroy. The drumbeat of war has grown louder since the beginning of this year, with civilians in the direct line of fire. From Lebanon to Myanmar, from Haiti to Sudan, people are being killed, displaced, wounded, raped, and cut off from food, water and healthcare. Hospitals are being attacked at alarming rates. Humanitarian colleagues are being killed and injured as they try to save lives.

The nature of warfare is also changing. The rapid spread of drones is reshaping the battlefield — extending the reach of armed actors, increasing the frequency of attacks, and exposing civilians, including aid workers, to danger in places once thought relatively safe. Danger is no longer confined to the frontlines. It is constant, diffuse, and increasingly unpredictable.

Layered onto this is a worsening global risk environment. Forecast El Niño conditions threaten to intensify droughts and floods across multiple regions. The Ebola outbreak in Central and East Africa is spreading in fragile and conflict-affected areas. And while the full consequences are still emerging, disruptions to the Strait of Hormuz could have far-reaching effects on the cost and delivery of aid.

We are adapting through the Humanitarian Reset: streamlining the way we work; promoting cash and voucher assistance; pushing for anticipatory action; and calling for people's voices to truly shape decisions, not just get heard and ignored.

But we need to be honest. We are not doing more with less. We are doing less with less. And that has real consequences for the people we serve.

As we enter the second half of the year, I urge you to focus on the people behind these numbers. For millions, this is not abstract. It is about survival. It is about dignity.

The 87 million lives campaign is a reminder that even in the toughest places, we can still save lives—one at a time. Member States have already stepped forward, providing funding earlier than usual. I urge others—individuals, organizations, companies—to do the same.

And we need a fundamentally different approach to protecting civilians, including aid workers. There is no point saving someone today if they are killed tomorrow.

Trends in crises and needs: civilians in the line of fire

In the first five months of 2026, crises grew and intensified in multiple countries, driving rising humanitarian needs.

By the end of May 2026, over 252 million people were in urgent need of humanitarian assistance and protection—an additional 13 million people since the 2026 Global Humanitarian Overview was launched in December 2025. Across these crises, women and girls faced heightened protection risks and barriers to accessing critical and life-saving assistance and services, while also playing critical roles as first responders, community leaders and providers of support within affected communities.

Situations of violence, including conflict—which remains the main driver of humanitarian need globally—escalated, with civilians and civilian infrastructure under direct attack, international humanitarian law repeatedly defied and people forcibly displaced from their homes. Conflict between States is at its highest level since World War II. In the Middle East, over 7,000 people have been killed, including 3,637 in Lebanon, 3,375 in Iran and 29 in Israel, while civilian infrastructure has come under attack by all parties to armed conflict across the region. In Lebanon, an estimated 1.4 million people require urgent life saving assistance and protection, as displacement becomes increasingly prolonged and pressures on host communities and essential services intensify. In Sudan, at least 160 children were killed and 85 maimed in the first three months of 2026, a shocking 50 per cent more than in the first quarter of 2025, while people continued to flee the horrifying war internally, and across borders as refugees. In Haiti, escalating violence has forced record levels of people from their homes, with 1.5 million people internally displaced by June, including over 300,000 in the capital, Port-au-Prince. In the Occupied Palestinian Territory, children have been shot, stabbed, beaten, and pepper-sprayed in the West Bank, with 70 children killed between January 2025 and mid-May 2026 (an average of one child killed each week), while in Gaza, more than 250 children were killed and 260 injured from the announcement of the ceasefire in October 2025 to May 2026. In Ukraine, May 2026 marked the highest monthly civilian casualty toll—at least 274 civilians were killed and 1,763 injured—since July 2025, while attacks with long-range weapons struck cities and towns far from the frontline. And when fighting re-erupted between Afghanistan and Pakistan in late-February 2026, it displaced over 100,000 people in Afghanistan, at a time when communities across the country were already struggling to cope with cross-border returns and rising hunger, which has hit women hardest and forced families to take desperate decisions, including selling their daughters into early marriage. Across several crises, refugees and internally displaced people began to return home, including in Afghanistan, Syria and Sudan. However, they often arrived to lack of functioning basic services, damaged houses and other challenges.

The human toll of drone warfare, which increased exponentially between 2020 and 2024, was one of the defining trends in armed conflict in early 2026, cheap and deadly. The expansion in drone use is fundamentally

reshaping the way wars are waged, with catastrophic consequences for civilians. In Sudan, armed drones caused 80 per cent of civilian deaths between January and April 2026, with at least 28 drone attacks harming markets and 12 drone attacks harming health facilities in that four-month period alone. In Ukraine, short-range drones killed 80 civilians and injured 481 in April 2026, the highest monthly civilian toll from this type of weapon since the start of the invasion. In the Russian Federation, infrastructure and civilian objects have been hit and civilians have died including as a result of drone attacks. In Colombia, there was a 445 per cent increase in attacks using weaponized drones between 2024 and 2025, with significant humanitarian consequences. Humanitarians are increasingly in the line of drone fire. In a single week in May, four humanitarian convoys were hit while carrying life-saving assistance to civilians in need in Ukraine. In March, an aid worker was killed in a drone strike on a residential building in the Democratic Republic of the Congo.

Conflict-related sexual violence, marked by extreme brutality, has continued to surge, overwhelmingly targeted against women and girls. Nearly 10,000 cases of conflict-related sexual violence were recorded in 2025—more than double the previous year's figure. In Haiti, armed groups use sexual violence to spread fear among, subjugate, and punish the population. In Myanmar, rape, gang rape and other forms of sexual violence are used as a tactic of war, including during village raids, at check points, in fields, homes and detention settings and online. In DRC, sexual violence against children has become entrenched, systemic and increasingly widespread. In Sudan, sexual violence continues to surge, with the number of women and girls requiring support after experiencing gender-based violence nearly doubling in two years and quadrupling since the start of the war three years ago.

The escalation of conflict in Iran and across the Middle East has far-reaching effects, causing global supply disruptions and rising humanitarian costs. The war has severely restricted shipping through the Strait of Hormuz, a critical chokepoint that carries about one fifth of global oil and gas and a significant share of fertilizer trade. The disruption has sharply increased fuel, transport and insurance costs, raising humanitarian operating expenses and constraining fertilizer supplies and represents the beginning of a systemic agrifood shock that could trigger a severe global food price crisis within six to 12 months. At the same time, the disruptions are significantly increasing the cost and complexity of humanitarian operations at a time when global humanitarian action is desperately under-funded, with longer shipping times, higher fuel and logistics costs, and growing constraints on supply chains reducing the efficiency and reach of response efforts. Evidence from partners, including WFP, IRC, IFRC, CARE, and organizations in Somalia indicate that rising fuel, transport and logistics costs are already increasing the cost of delivering assistance, which may translate into fewer people reached, smaller rations and reduced service packages.

Shadia*, escaping the siege of El Fasher



For many people, siege is an ancient word – something from history books. But for Shadia, it is painfully real. She lived through the 500-day siege of El Fasher, trapped with thousands of others in a city cut off by war, with dwindling food and medicines, with troops surrounding them. The farms around the city were emptied through fear and violence. Those who tried to leave in search of food or to bring supplies back risked being killed, detained, or raped.

Shadia remained in El Fasher as the city starved and collapsed around her, until one desperate attempt to find vegetables outside the city ended in rape.

After her assault, she escaped to Tawila by foot, more than 50km away. Her husband did not escape. When El Fasher fell in October 2025, she was finally reunited with her children. Today, she lives with them in a small, crowded tent in a displacement camp.

Across Sudan, over 12 million people, mostly women and girls, are at risk of gender-based violence – an 80 per cent increase since 2024. In Tawila, Shadia and other survivors can access emergency clinical treatment, receive treatment for their injuries, and support from trained counsellors and other women who have endured similar violence.



Tawila, North Darfur, Sudan

Shadia, a displaced mother from El Fasher, relates surviving detention and assault before fleeing to Tawila. Her story reflects widespread abuses reported during the RSF takeover that forced mass displacement across North Darfur.

UNOCHA/Giles Clarke.

These new pressures come when acute food insecurity and malnutrition are already alarmingly high. Hunger has doubled over the past decade, with 266 million people across 47 countries facing crisis-level conditions in 2025, while an estimated 35.5 million children and about 9.2 million pregnant and breastfeeding women were acutely malnourished across 23 countries/territories, including just under 10 million children facing life-threatening severe acute malnutrition. Hunger is primarily driven by conflict, and is also highly concentrated: ten countries—Afghanistan, Bangladesh, the Democratic Republic of the Congo, Myanmar, Nigeria, Pakistan, South Sudan, Sudan, Syrian Arab Republic, and Yemen—accounting for about two thirds of the global total. Across crises, women and girls are often the last to eat and the least likely to access scarce resources. Rising hunger is contributing to negative coping strategies, including child marriage, transactional sex, school dropout among girls, and increased protection risks. Following the unprecedented declaration of two simultaneous famines in 2024—in parts of Sudan and in Gaza in the Occupied Palestinian Territory—people are projected to face Catastrophe (IPC/CH Phase 5) levels of food insecurity in 2026 in Sudan (207,000 people) Yemen (41,000 people), South Sudan (28,000 people), Nigeria (15,000) people and the Occupied Palestinian Territory (Gaza Strip, 2,000), while the Burhakaba district of Somalia was declared to be at risk of famine in May 2026. If hostilities in

the Middle East persist, around 45 million more people could fall into acute hunger, with the largest projected increases in Africa and Asia.

Meanwhile, climate shocks continued to wreak havoc on people's lives in the first half of 2026. In December 2025, severe and prolonged flooding affected large parts of southern and central Mozambique, affecting around 724,000 people and driving humanitarian needs requiring a response in 2026. Rivers burst their banks, displacing communities and damaging or destroying homes, health facilities, water systems and other critical infrastructure. Then, in early 2026, Madagascar was hit in rapid succession by Cyclone Fytia on 31 January and Intense Tropical Cyclone Gezani on 10 February, causing widespread devastation across the northern, western, and eastern regions of the country. The disaster significantly worsened an already fragile humanitarian situation, with more than 681,000 people affected and 632,000 now requiring urgent assistance. In Cuba, severe fuel shortages due to external measures compounded the needs triggered by Hurricane Melissa (October 2025), affecting access to water, health care, sanitation, education and food, while also limiting partners' ability to distribute assistance already in-country. In Somalia, a severe and prolonged drought—combined with insecurity, cuts to humanitarian assistance and the ripple effects of war in the Middle East—has caused a rapidly intensifying emergency.

Olena, surviving on the frontlines of war in Ukraine



Kherson, Ukraine

UNOCHA/Viktoriiia Andriievskia

"This cannot be called life," said 80-year-old Olena, as she reflected on the past four years of the ongoing war in Ukraine.

"It is not life when every day, every moment, there is shooting and you are frightened. It feels like there is a 'hunt for people' with drones, a human safari," - she said. "Is that life? No."

And yet, Olena wakes up to meet every morning. She makes breakfast for herself and her daughter. She waits for news from her family. It is her daily ritual which she helps her hold on.

For families in Kherson and other front-line communities, survival has become an act of quiet endurance.

Before Russia's full-scale invasion in February 2022, Olena and her husband had built a quiet, steady life in a close-knit neighbourhood in Kherson. But all of that changed, when parts of Kherson were occupied, including their

neighbourhood. Electricity, water, mobile communication — everything stopped. Checkpoints appeared. Armed men patrolled the streets. People stayed indoors, speaking in whispers, measuring each movement against fear. Even after Russian forces left, Kherson never truly left the front line: artillery and drones can reach within minutes. And in early-2023, Olena's home was damaged by shelling.

Today, Olena lives in a rented apartment in Kherson with her daughter. Their pension barely covers expenses. For families living within range of artillery and drones, war is measured in sleepless nights and shattered homes.

"I don't mourn the loss of property," Olena's daughter said. "I mourn the people the war has taken. Everything else can be rebuilt. But lives cannot be returned."

Against a backdrop of sustained warming and increasing climate extremes, El Niño conditions, **predicted to start in June 2026, are set to **increase the risk of extreme weather over the coming months**. Although no two events are the same, the 2026 El Niño could increase the risk of heat waves, severe droughts and torrential rains in multiple parts of the world.**

On top of conflict and climate crises, a new Ebola outbreak was declared in May 2026 in both the Democratic Republic of the Congo (DRC) and Uganda. The resurgence of the virus in DRC is unfolding amid overlapping crises that heighten transmission risks, including conflict, mass displacement, and limited access to healthcare. Women are disproportionately exposed to infection through their caregiving and frontline health roles, while disruptions to maternal health, sexual and reproductive health and gender-based violence services are impacting women and girls

uniquely. Clashes are driving mass displacement, pushing exposed contacts into overcrowded camps, and cutting off critical epidemiological corridors. Attacks on health facilities and threats to frontline workers are making case detection, contact tracing and safe burials highly complex. This is the 17th Ebola outbreak in the DRC, emerging only a few months after the end of the last epidemic in December 2025. The epidemic was declared a Public Health Emergency of International Concern (PHEIC) on 17 May 2026, underscoring the urgency of the situation. There is currently no vaccine and no approved treatment for this strain of the Ebola virus, the Bundibugyo virus.

Tawila, North Darfur, Sudan

Young displaced girls collect firewood near Tawila, often traveling into unsafe areas between armed groups. Aid groups warn these daily tasks for firewood and water expose women and girls to heightened risks of violence and exploitation in frontline areas during Sudan's ongoing war.

UNOCHA/Giles Clarke



Progress in Overall GHO and Prioritized GHO (June 2026)

Plan Type	Plan Name	GHO Required	GHO Funding	GHO Coverage	GHO Required (Prioritized)	GHO Funding (Prioritized)	GHO Coverage (Prioritized)
HNRP	 Afghanistan	\$1,714,181,496	\$269,170,017	15.70%	\$465,232,523	\$246,996,503	53.09%
HNRP	 Burkina Faso	\$658,486,359	\$94,505,966	14.35%	\$350,362,294	\$94,100,604	26.86%
HNRP	 Cameroon	\$318,976,221	\$75,801,113	23.76%	\$268,368,745	\$73,611,850	27.43%
HNRP	 Colombia	\$286,966,367	\$160,276,402	55.85%	\$286,966,367	\$143,930,699	50.16%
HNRP	 Haiti	\$880,327,426	\$199,123,342	22.62%	\$680,379,644	\$199,060,842	29.26%
HNRP	 Mali	\$577,866,412	\$94,903,911	16.42%	\$329,221,931	\$94,391,189	28.67%
HNRP	 Mozambique	\$534,342,584	\$139,094,280	26.03%	\$265,305,908	\$133,667,941	50.38%
HNRP	 Myanmar	\$889,600,798	\$339,836,193	38.20%	\$521,070,982	\$256,314,094	49.19%
HNRP	 Niger	\$675,619,589	\$93,016,189	13.77%	\$96,492,592	\$79,290,447	82.17%
HNRP	 Nigeria	\$516,399,423	\$219,229,972	42.45%	\$516,399,424	\$207,433,052	40.17%
HNRP	 République Centrafricaine	\$264,097,388	\$57,034,656	21.60%	\$214,778,613	\$56,861,449	26.47%
HNRP	 République Démocratique du Congo	\$1,403,721,718	\$481,041,871	34.27%	\$936,973,669	\$460,149,723	49.11%
HNRP	 Somalia	\$852,000,000	\$157,688,614	18.51%	\$349,999,999	\$154,887,829	44.25%
HNRP	 South Sudan	\$1,463,046,056	\$364,823,658	24.94%	\$1,042,108,966	\$332,343,357	31.89%
HNRP	 Sudan	\$2,866,228,593	\$642,474,512	22.42%	\$2,143,603,358	\$641,240,104	29.91%
HNRP	 Syrian Arab Republic	\$2,922,184,830	\$537,380,945	18.39%	\$1,539,672,828	\$533,265,796	34.64%
HNRP	 Tchad	\$986,069,627	\$240,469,159	24.39%	\$620,188,576	\$227,398,391	36.67%
HNRP	 Ukraine	\$2,303,961,975	\$986,811,583	42.83%	\$2,049,194,156	\$980,010,282	47.82%
HNRP	 Venezuela	\$632,191,972	\$99,824,197	15.79%	\$244,809,343	\$99,812,228	40.77%
HNRP	 Yemen	\$2,162,563,336	\$316,777,743	14.65%	\$1,596,138,530	\$316,777,743	19.85%
HNRPs Total		\$22,908,832,170	\$5,569,284,323	24.31%	\$14,517,268,448	\$5,331,544,123	36.73%
FA	 Escalation of Hostilities in the OPT Flash Appeal 2026	\$4,064,305,808	\$564,781,162	13.90%	\$4,064,305,808	\$564,781,162	13.90%
FA	 Lebanon Flash Appeal 2026	\$639,875,890	\$185,906,742	29.05%	\$639,875,890	\$184,314,002	28.80%
FA	 Madagascar Appel Éclair Cyclones Fytia et Gezani 2026	\$67,953,475	\$36,287,022	53.40%	-	-	-
FAs Total		\$4,772,135,173	\$786,974,926	16.49%	\$4,704,181,698	\$749,095,164	15.92%

Regional Response Plans and Other Plans

Plan Type	Plan Name	GHO Required	GHO Funding	GHO Coverage	GHO Required (Prioritized)	GHO Funding (Prioritized)	Cove (Prioriti
REG	Islamic Republic of Iran Flash Refugee Response Plan March - May 2026	\$80,000,000	\$37,599,080	47.00%	\$80,000,000	\$37,599,080	47.00%
REG	Regional Migrant Response Plan for Horn of Africa to Yemen and Southern Africa 2026	\$52,990,581	\$30,574,322	57.70%	\$25,432,136	\$30,574,322	120.22%
REG	Rohingya Humanitarian Crisis Joint Response Plan 2026	\$710,508,363	\$271,942,273	38.27%	\$610,688,472	\$271,942,273	44.53%
REG	Sudan Emergency: Regional Refugee Response Plan 2026	\$496,917,924	\$168,673,981	33.94%	\$353,306,192	\$168,673,981	47.74%
REG	Syrian Arab Republic Regional Refugee and Resilience Plan (3RP) 2026	\$2,800,721,613	\$150,266,309	5.37%	\$2,800,721,613	\$150,266,309	5.37%
REG	Uganda Regional Refugee Response Plan 2026	\$828,700,000	\$70,480,477	8.50%	\$423,000,000	\$70,480,477	16.66%
REG	Venezuela Regional Refugee and Migrant Response Plan (RMRP) 2026	\$763,092,338	\$76,779,790	10.06%	\$450,739,267	\$76,779,790	17.03%
REGs Total		\$5,732,930,819	\$806,316,232	14.06%	\$4,743,887,680	\$806,316,232	17.00%
Other	 Cuba Plan of Action 2026 – Response to Hurricane Melissa	\$81,755,350	\$16,689,415	20.41%	\$81,755,350	\$15,036,659	18.39%
Other	 Pakistan Floods Support Plan 2026	\$64,943,910	\$20,626,098	31.76%	-	-	-
Other	 Sri Lanka Humanitarian Priorities Plan Cyclone Ditwa 2026	\$35,300,000	\$19,651,492	55.67%	-	-	-
Other	 Viet Nam 25-26 Joint Response Plan Multiple Typhoons and Floods 2026	\$61,132,366	\$3,155,142	5.16%	-	-	-
Others Total		\$243,131,626	\$60,122,147	24.73%	\$81,755,350	\$15,036,659	18.39%
	GHO 2026 (Plan not yet defined)	-	\$995,535,221	-	-	\$995,535,221	-
		\$33,657,029,788	\$8,218,232,849	24.42%	\$24,047,093,176	\$7,897,527,399	32.84%

Humanitarians in action in 2026: Delivering against the odds

As new crises emerged and existing crises deepened, humanitarians adapted their planning in the first half of 2026, with the number of people targeted rising to 143 million, and the number of people prioritized increasing to nearly 89 million, by mid-year. In January 2026, humanitarians aimed to reach 135 million people, of whom 87 million living in areas with the most severe needs were hyper-prioritized. In the first five months of the year, however, needs rose in Cuba, Lebanon, Madagascar and Mozambique, requiring humanitarian partners to expand their planning. As a result, an additional 8 million people are being targeted, including an additional 1.9 million people prioritized, at the mid-year point.

In the face of dramatic funding cuts and escalating attacks, humanitarians prioritized reaching people facing the most urgent needs, grounded in the principle of impartiality. This is reflected in the growing concentration of humanitarian assistance in areas with the highest levels of need. Partners provided an estimated 45.2 million people¹ with at least one form of humanitarian assistance and protection by the end of April 2026². Some 25.4 million of the people reached (58 per cent) were in the geographic areas where needs were most severe, compared to an average of 43 per cent in 2025. The total includes over 5.3 million refugees assisted through refugee responses³. However, while the hyper-prioritized response has delivered necessary focus at a time of immense resource constraints, these numbers should be placed in context: only around one in six people in need were reached in the first months of 2026 (45.2 million out of 252 million), with many people receiving only partial assistance or reduced service packages.

In every crisis, communities were at the forefront of the response. In Sudan, for example, women's response rooms (WRRs) evolved organically on the sidelines of community kitchens, initially so that women and girls could gather safely and provide each other moral support. Today, they provide specific assistance requested by women and girls, such as reproductive health information and female sanitary items, as well as psychosocial support and acute malnutrition screening. Globally, Village Savings and Loan Associations (VSLAs) members have adapted the use of their social funds—usually reserved for social events and activities—to respond to crisis-related needs, including in South Sudan. In the Central African Republic, the country-based pooled fund established a dedicated window to create allocation rounds reserved for local and national NGOs and women-led organizations.

Despite the immense constraints they faced, humanitarian partners saved lives, delivered essential services, and assisted people impacted by crises around the world:

- **Food security and nutrition partners worked intensively to meet immense needs amidst declining resources.** In Gaza, Occupied Palestinian Territory, 1.6 million meals were prepared and delivered daily by 26 partners, providing critical sustenance for families facing extreme shortages. In South Sudan, 1.4 million people have been reached with multi-sectoral life-saving delivery of food, health care, safe drinking water and shelter. In Venezuela, 350,000 people have received food security support in 2026 to date, as part of the ongoing multi-sectoral response, which reached 739,000 people by the end of April.
- **Humanitarian workers and community volunteers delivered critical protection services, ensuring that people—including women, girls, people with disabilities and older people—received the tailored care, safety, and support they required.** In the Democratic Republic of the Congo, some 572,000 people were assisted by 178 protection partners, the majority being local and national organizations, including children associated with armed groups supported for reintegration, women and girl survivors of violence receiving psychosocial and medical support, and people reached with explosive ordnance risk education. As part of the Humanitarian Reset, the Protection Cluster was consolidated, bringing together expertise across the sector's full membership, including child protection, gender-based violence and mine action.
- **Partners delivered essential health and water, sanitation and hygiene (WASH) services, including sexual and reproductive healthcare.** In Nigeria, humanitarians stepped up efforts to contain the rapidly expanding outbreak in Borno State, including through the expansion of WASH services, early detection, and case management. In Bangladesh, nearly 79,000 people received sexual and reproductive health services.
- **Humanitarians provided shelter and critical supplies and improved conditions for people enduring forced displacement.** In Sudan, the largest internal displacement crisis globally, more than 393,000 people received essential non food items in the first half of the year, helping families meet basic needs amid extreme violence. In Ukraine, over 780,000 people received winter energy assistance and insulation support, primarily delivered through cash-based assistance. Under the Reset, a new Global Shelter, Land and Site Coordination Cluster was formed, bringing together shelter assistance, site coordination, and housing, land and property support in a new place-based, people-centred approach.
- **Recognizing the priorities set by children themselves, education partners supported access to learning,** including through online platforms, educational packages and safe spaces. In Syria, partners are supporting access to safe, inclusive and continuous learning for

1. Across at least 38 countries through 32 out of 34 plans included in the 2026 GHO as of 12 June.

2. Except for Lebanon where response data covers January to May 2026.

3. This is based on the data reported against five regional plans – namely Joint Response Plan for the Rohingya Humanitarian Crisis; Sudan Regional Refugee Response Plan; Regional Refugee and Resilience Plan for the Syria Crisis; Uganda Country Refugee Response Plan; and Regional Refugee and Migrant Response Plan for Refugees and Migrants from Venezuela.

displaced and returning children: out of 3.8 million people targeted, 36 per cent are displaced students, 14 per cent are previously displaced students and six per cent are students who were refugees abroad and have returned to their homes in Syria.

Through the Humanitarian Reset, partners continued their efforts to make responses more accountable, more efficient, and closer to people affected by crises:

- **Across responses humanitarian partners and leaders worked to ensure accountability to affected people drives decision-making, determined to ensure that this is not just about listening, but about acting on what is heard.** In Sudan, a multi-channel interagency complaints and feedback mechanism (CFM) was established to advance collective accountability. Cameroon has also established a joint collective feedback mechanism, with partner training on feedback management and data protection, along with a perceptions tracking exercise conducted primarily with volunteer groups in affected communities. Mozambique has Linha Verde, an established interagency feedback mechanism that combines multiple channels, with direct access for Cluster Coordinators to follow up on issues within their sectors, and monthly published reports.
- **Cash and voucher assistance—including multi-purpose cash—has decreased significantly due to funding cuts, but where it was provided, it delivered people impacted by crises with fast, efficient and dignified support.** In Sudan, when households received cash, they were able to meet multiple needs simultaneously across sectors and prioritize according to each family's priorities, with measurable improvements in basic needs, food security and coping capacities. In Ukraine, approaches such as community-based cash transfers delivered by civil society approaches, enabled responses in hard-to-reach and underserved communities. New global research highlights the efficiencies of cash-based responses, particularly when delivered by local and national partners.
- **Anticipatory action was leveraged to act early and save both lives and money.** In March, for example, collective anticipatory action frameworks for drought were activated in Honduras, Guatemala and El Salvador after forecasts indicated significant rainfall deficits during the upcoming Primera agricultural season.
- **Shared services played a critical role in enabling timely, effective and efficient responses.** The United Nations Humanitarian Air Service (UNHAS) delivered people and supplies to the world's most challenging and remote

locations. When Ebola broke out in the DRC, UNHAS transported responders and delivered cargo, facilitating a rapid scale-up in the response. Likewise, logistics coordination keeps aid moving even when access constraints rise. In Lebanon, where roads were damaged, debris and unexploded ordinance were pervasive, and key corridors were disrupted, ten coordinated convoys reached hard to reach areas with life-saving assistance in one month. Connectivity is increasingly recognized as essential to effective humanitarian response. In Yemen, emergency telecommunications kept 2,500 humanitarian staff connected across 49 organizations, but coverage reduced from 19 to 5 locations due to resource constraints. Through the Reset, the Logistics and Telecommunications Cluster was created to support a faster, lighter, more accountable, and more impactful humanitarian response, and has already been activated in Lebanon.

- **In several crises, direct participation of crisis-affected women and women-led organizations in humanitarian decision-making was enhanced through dedicated Women's Advisory Groups,** including in Afghanistan, Sudan and Syria. These mechanisms provide a platform for women leaders and women-led organizations to bring community priorities, protection concerns and operational recommendations directly to humanitarian leadership. Efforts were also made to strengthen the influence of local actors on humanitarian financing decisions, including through the participation of women-led organizations on 18 out of 21 Country-Based Pooled Fund Advisory Boards, helping to inform funding priorities and allocation decisions and advance commitments to localization.

This collective, global, humanitarian response has been enabled by communities standing with their neighbours, individuals and organizations stepping-up to donate to people in crisis, and humanitarian donors funding vital humanitarian action. Through the 87 million lives campaign and intensive individual and collective mobilization by humanitarian organizations, humanitarian financing was significantly stronger by the end of May 2026 compared to the same time in 2025. Contributions to the Global Humanitarian Overview reached US\$8.21 billion, a 69 per cent increase from the \$4.86 billion received in 2025 by end-May and slightly above 2024 levels.

However, no amount of effort or resource mobilization can substitute for the seismic contraction of humanitarian action caused by funding cuts in 2024 and 2025.

Democratic Republic of Congo (DRC) – Ebola outbreak in May 2026



Ituri Province, Democratic Republic of the Congo

Marléne*, a 25-year-old mother, brings her two-month-old daughter, Deborah*, to a health facility for vaccination.

Save The Children/Patou Ndombi

There is currently no specific vaccine or treatment available for this Ebola strain and hundreds have already lost their lives, including children. This is unfolding in a context where children are already facing one of the world's most severe humanitarian crises, with ongoing conflict, mass displacement, and fragile health systems.

Marléne*, a 25-year-old mother, brought her two-month-old daughter, Deborah*, to a health facility for vaccination. As a breastfeeding mother, Marlène* was especially worried about how easily Ebola can spread. She fears that if she were to become infected, she could pass the virus to Deborah*. "This is what worries us the most" she says, "how we can protect our children."

At the clinic, Marlène* has received guidance on preventive measures, including frequent handwashing and maintaining physical distance, which she now tries to follow carefully in her daily life.

Since the Ebola outbreak was declared, humanitarian organizations have scaled up their support, working to control the outbreak, assisting families impacted, engaging with communities to foster acceptance and risk communication, and centering protection against sexual exploitation and abuse. In consultation with the WHO Director-General and inter-agency partners, the Emergency Relief Coordinator has activated the [IASC System-Wide Scale-Up Protocol for the Control of Infectious Disease Events](#) to ramp-up the response.

Uganda | Community feedback strengthens targeting after food assistance cuts.



Uganda

Gloria works at a food distribution point help desk in Kiryandongo refugee settlement informing and registering claims from refugees. Community Feedback Mechanisms (CFMs) give people a voice—through helplines, help desks, and community meetings—ensuring their questions, concerns, and feedback are heard, tracked, and responded to.

WFP/Badre Bahaji

In Uganda, funding cuts forced humanitarian partners to reduce humanitarian assistance, with more than one million refugees phased out of food assistance. In order to determine who should be prioritized for assistance, partners undertook a nationwide food assistance categorization exercise. However, given the consequences, serious concerns were raised by refugees. To listen to the community, humanitarian partners established appeals and feedback channels, receiving over 60,800 appeals covering 390,000 people. Community feedback highlighted exclusion risks for people with disabilities, chronic illness and other vulnerabilities, leading to reclassification of more than 64,000 people and informing improvements to future targeting.

* Names have been changed to protect identities. Marlène's story was compiled and shared by Save the Children.

GHO mid-year results (2026)

Plan Name	People Targeted	Projected People Prioritized (PIN under Severity 4&5)	People Reached (4&5)	People Reached (Total)	% People Reached (4&5) to People Reached (Total)	% People Reached (4&5) to Projected People Prioritized (PIN 4&5)	% People Reached (Total) to People Targeted	Reporting Period
 Afghanistan	17,479,657	4,574,408	1,015,572	4,665,633	22%	22%	27%	January - March
 Burkina Faso	2,651,923	1,598,576	333,385	786,865	42%	21%	30%	January - April
 Cameroon	1,880,879	1,753,003	121,354	172,561	70%	7%	9%	January - March
 Central African Republic	1,265,483	183,797	178,908	214,941	83%	97%	17%	January - March
 Chad	3,352,810	1,125,759	371,916	657,490	57%	33%	20%	January - March
 Colombia	2,656,763	2,656,763	183,341	308,139	59%	7%	12%	January - April
 Democratic Republic of the Congo	7,313,763	5,864,510	1,418,677	3,094,992	46%	24%	42%	January - March
 Haiti	4,162,202	2,097,190	658,647	1,396,217	47%	31%	34%	January - March
 Mali	3,808,385	353,179	20,709	223,428	9%	6%	6%	January - March
 Mozambique	1,745,582	919,494	461,009	740,749	62%	50%	42%	January - April
 Myanmar	4,868,096	3,286,538	832,133	1,599,923	52%	25%	33%	January - March
 Niger	1,983,640	401,349	48,370	615,341	8%	12%	31%	January - March
 Nigeria	2,527,178	1,849,891	429,793	552,254	78%	23%	22%	January - March
 oPt	2,970,096	2,970,096	777,915	777,915	100%	26%	26%	January
 Somalia	2,430,751	1,983,877	503,088	1,283,488	39%	25%	53%	January - March
 South Sudan	4,343,435	3,850,139	1,370,253	2,285,834	60%	36%	53%	January - April
 Sudan	20,419,163	18,443,114	6,448,629	9,004,455	72%	35%	44%	January - April
 Syria	8,561,766	-	-	-	-	-	-	-
 Ukraine	4,118,954	3,615,753	1,689,745	3,369,179	50%	47%	82%	January - April
 Venezuela	5,453,591	1,307,781	196,470	739,273	27%	15%	14%	January - April
 Yemen	12,004,155	9,426,659	831,398	1,331,045	62%	9%	11%	January - April
 Cuba	2,000,000	-	1,000,000	1,000,000	100%	-	50%	January - April
 Lebanon	1,390,000	-	680,000	680,000	100%	-	49%	January - May
 Madagascar	454,975	-	-	356,000	-	-	78%	January - April

Plan Name	People Targeted	Projected People Prioritized (PIN under Severity 4&5)	People Reached (4&5)	People Reached (Total)	% People Reached (4&5) to People Reached (Total)	% People Reached (4&5) to Projected People Prioritized (PIN 4&5)	% People Reached (Total) to People Targeted	Reporting Period
 Pakistan	1,900,000	-	-	226,723	-	-	12%	January - April
 Sri Lanka	658,370	-	-	574,557	-	-	87%	January - April
 Viet Nam	658,761	-	-	99,577	-	-	15%	January - April
Horn of Africa to Yemen and Southern Africa (MRP)	868,751	-	135,246	140,290	96%	-	16%	January - March
Islamic Republic of Iran (RRP)	2,800,000	-	-	-	-	-	-	-
Rohingya JRP	1,557,361	-	1,246,761	1,321,625	94%	-	85%	January - March
Sudan Regional RRP	2,557,885	-	1,672,859	1,672,859	100%	-	65%	January - March
Syria 3RP	8,565,035	-	584,741	584,741	100%	-	7%	January - March
Uganda Country RRP	2,225,443	-	605,396	1,672,660	36%	-	75%	January - March
Venezuela (RMRP)	1,524,936	-	62,086	62,086	100%	-	4%	January - March



Mogadishu, Somalia

Hussein, an amputee living in a displacement site in Mogadishu, struggles to meet basic needs without family support. “The number of people keeps increasing, and this means more starvation. This morning I did not even have breakfast. We need people to visit and see our problems with their own eyes.”

UNOCHA/ Farhasaad Shahid

The consequences of inaction: what happens when global solidarity dissipates & aid comes under attack

Despite humanitarian best efforts, underfunding in 2026 will leave millions of people without essential assistance, while attacks harming aid workers and assets prevent civilians from accessing the assistance and protection they need to survive. These cuts and attacks are not isolated disruptions; they represent the rupturing of a lifeline for people in crisis. For women and girls, the cost of inaction is particularly acute. It means giving birth without skilled care, surviving violence without access to support services, withdrawing daughters from school, losing access to reproductive healthcare, and facing heightened risks of exploitation, abuse and child marriage. It also means the erosion of community-based networks and women-led organizations that often provide the last remaining lifeline when formal systems falter.

Funding cuts have forced humanitarian to make excruciating decisions regarding prioritization, cutting a lifeline for tens of millions of people in need. Based on collective needs analysis and intense discussions by country teams, assistance in 2026 is focused on those facing the most severe needs. However, while necessary, this leaves tens of millions of people without any hope of support. In [Somalia](#), for example, alarming food insecurity has been compounded by declining humanitarian assistance: only 17 percent of people in need received humanitarian food security support in January 2026 while 125 treatment sites for severe acute malnutrition and 360 sites offering treatment for moderate acute malnutrition have been closed due to under-funding.

The consequences are already visible across crises, in lives lost, services reduced, and protection weakened:

- **Resourcing constraints have weakened healthcare systems that detect dangerous pathogens, prepare countries for and prevent outbreaks, and guide vaccine decisions.** Somalia has been forced to close 25 health facilities because of funding gaps, cutting off essential services for 700,000 people. In [South Sudan](#), more than 1.35 million people in Jonglei State lost access to health services due to damage, looting, and service disruptions.
- **Unsafe water, sanitation and hygiene services (WASH) are causing disease and despair in multiple countries.** In [Gaza, Occupied Palestinian Territory](#), collapsed sanitation systems, overflowing sewage, extreme overcrowding, and widespread rodent and insect infestations are exposing civilians to disease, suffering and untenable living conditions amid soaring temperatures.
- **Funding gaps are forcing millions of people to survive in unsafe, degrading, and increasingly dangerous living conditions.** In the Horn of Africa—particularly Ethiopia—the upcoming rainy season is increasing concerns about aging, temporary shelters located in areas exposed to strong winds, flooding, and poor site conditions. Chad, hosting over 927,000 Sudanese refugees, is struggling to relocate families into organized sites where protection services are already overstretched.
- **Protection and education systems are collapsing in multiple contexts due to lack of resources.** In Somalia, 86 per cent of GBV facilities have shut down, potentially

leaving more than 50,600 women and girls without specialized services in 2026. In Chad, severe funding shortfalls are forcing the closure of eight Women and Girls Safe Spaces and threatening the sustainability of 12 One Stop Centers, leaving survivors of gender-based violence without clinical care, psychological support or safe entry points. In Colombia, local organisations have been forced to scale back drastically. Between 30 per cent and 50 per cent of Education in Emergencies programmes have been reduced or completely suspended, directly impacting the provision of safe learning spaces, delivery of school materials and e-learning kits and other essential elements of education responses in crisis-hit regions. Households experiencing cuts in assistance are 64 per cent more likely to withdraw children from school in order to work or beg and more than twice (2.1 times) as likely to experience family separation.

At the same time, an erosion of protection, including flagrant violations of international humanitarian law, are exposing humanitarian and healthcare workers to greater danger and severely hampering efforts to provide assistance and care.

- **There has been a collapse of protection for aid workers.** In 2025, at least 326 humanitarians were recorded as killed across 21 countries (an average of almost one each day), bringing the total number of humanitarians killed in three years to over 1,010. Of those over 1,000 deaths, more than 560 were in Gaza and the West Bank, 130 in Sudan, 60 in South Sudan, 25 in Ukraine and 25 in the DRC. Member States' efforts to investigate and bring perpetrators to justice when death and injury result from apparent violations remain grossly inadequate. This pervasive impunity undermines the protection of aid workers across crises.
- **Access constraints severely hampered humanitarian response in multiple countries.** In the Occupied Palestinian Territory, aid delivery remained limited by crossing restrictions, denied or delayed movement approvals, fuel shortages, and disruptions to supply chains. In Somalia, fighting was the primary cause of access restrictions in the first quarter, including hostilities, inter-clan clashes, political and election-related tensions that hindered humanitarian delivery by restricting the movement of personnel and supplies amid prepositioning for the drought response.
- **Attacks harming healthcare personnel and facilities have become a defining feature of conflicts,** despite clear obligations under international humanitarian law and the 10th anniversary of UN Security Council Resolution

2286, which was adopted in May 2016 to advance the protection of medical care in armed conflict.. From 28 February to 4 May, an average of 5 attacks per day on health care were reported in 2026, compared with 3.5 attacks per day during the same period in 2025. In Ukraine, more than 3,000 attacks were recorded on the health-care system in 1,534 days of war, from primary health care centres to maternity hospitals, and from ambulance teams to pharmaceutical warehouses. In Sudan, attacks against hospitals, clinics, ambulances, patients and health workers since April 2023 have killed more than 2,000 civilians. In Lebanon, almost 190 attacks on healthcare were verified, killing 128 healthcare workers and injured 332 others, were recorded in just three months in 2026. In the Democratic Republic of the Congo, attacks on healthcare facilities have deprived more than 55,000 people of medical care, displacing thousands and interrupting ongoing response efforts.

For millions of people, the consequences of global inaction are neither abstract nor in the future; they are already here, and they are immediate and they are life-threatening. Humanitarians therefore have two, straightforward, asks of the global community, including Member States, individuals, organizations, companies and parties to conflict:

- **Protect civilians in armed conflict, including the aid workers striving to assist them and the medical mission.** Uphold international humanitarian law consistently, not only with words, but with concrete actions.
- **Fully fund the hyper-prioritized Global Humanitarian Overview immediately,** while striving to fund the entire GHO by the end of 2026.

Somalia – choices made in faraway capitals reach Faadumo in Mogadishu



Mogadishu Feynus Health Centre, Somalia

Faadumo Sheikh Ahmed, 26, stands with her children at a displacement site in Mogadishu. She fled prolonged drought after crop failure, rising water prices, and the loss of livestock.

UNOCHA

When the Strait of Hormuz closes and aid budgets are cut in distant capitals, the consequences arrive in a displacement camp in Somalia's capital, Mogadishu – and in the lives of women such as Faadumo Shiekh Ahmed.

Faadumo arrived in Mogadishu with nothing. No food, no money, no possessions – only her seven children, two of them sick. She had walked for hours from Wanlaweyn District in Lower Shabelle Region, where the rains had failed for two years running. Her livestock had died and water prices had risen beyond anything she could afford. A stranger eventually gave her a ride to the capital. Someone told her she might find help there.

"We fled Wanlaweyn—we had nothing to carry," she said. Her three-year-old daughter has been malnourished for six months. Her nine-month-old son needs supplements she cannot access.

For a country that imports more than 90 per cent of its essential commodities and roughly 30 per cent of its fertilizer from the Persian Gulf region, the ripple effects have

been immediate and severe. Fuel prices in Somali markets have more than doubled – from US\$0.60 to \$1.50 per litre. Transport costs for humanitarian operations have doubled in turn. Water prices across drought-affected districts have increased by 200 to 300 per cent,

"Water prices have gone up and we could not afford it. We used to rely heavily on rains. In the past two years, there were no rains. We could not cultivate our land. Our livestock died," said Faadumo.

Faadumo's story is not an isolated one. Across Somalia, 6.5 million people now face hunger – double the number from August 2025. Global aid cuts have hit Somalia hard. The humanitarian community—more than 300 partners—is responding with the funding it has. But it is not enough. Without urgent additional support, more programmes will shut down entirely.



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